

QBITEL BRIDGE

HEALTHCARE PITCH Q&A GUIDE

65 Questions Across 10 Sections -- Objection Handling Included

65

Total Q&As

10

Sections

7

Hard Objections

8

Competitive Qs

Sections: Clinical Safety * Device Integration * FDA * HIPAA * EHR * Network * Ops * Procurement * Objections * Con

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CLINICAL & PATIENT SAFETY*8 Questions -- Addressing safety concerns for clinical environments***Q1 Will QBITEL Bridge interfere with life-critical device communications?**

A: No. Bridge operates as a transparent network overlay. All clinical traffic passes through with sub-millisecond overhead -- well within any clinical latency budget. We have validated on infusion pumps, ventilators, patient monitors, and cardiac devices. Emergency alert transmissions are always given highest priority and are never buffered or delayed.

Q2 What happens if Bridge itself fails? Will patients be at risk?

A: Bridge is deployed in fail-open mode for all life-critical device classes. If the Bridge node becomes unavailable, traffic automatically bypasses the encryption layer and flows in its original state -- protecting patient safety above all else. For non-critical devices, fail-secure mode is configurable. Our 99.999% uptime SLA is backed by redundant hardware deployment.

Q3 How does Bridge handle real-time vital sign alerts?

A: Critical clinical alerts (arrhythmia detection, SpO2 alarms, ventilator pressure alerts) are classified as Priority 0 traffic. They bypass all scheduling optimization, are encrypted in under 1ms, and delivered immediately. The Battery-Aware Scheduler never applies to Priority 0 traffic regardless of device battery state.

Q4 Does Bridge affect medical device accuracy or calibration?

A: Bridge operates exclusively at the network communication layer -- it never modifies device firmware, sensor data, or measurement logic. GSDF calibration on imaging workstations is preserved. Diagnostic accuracy is mathematically impossible to affect because Bridge touches only the encrypted transport envelope, never the clinical data payload.

Q5 How does Bridge interact with clinical alarm management systems?

A: Bridge integrates with clinical alarm management platforms (Rauland Responder, Vocera, Ascom) via their native APIs. Alarm events are encrypted in transit and delivered with guaranteed latency less than 1ms. Bridge enhances alarm security by authenticating alarm sources -- preventing spoofed clinical alarms, which are a documented attack vector.

Q6 Can Bridge protect implantable device programmer communications?

A: Yes. Implantable device programmer communications (pacemaker, ICD, neurostimulator programmers) operate in Protocol Proxy Mode. The programmer communicates normally with the implant; Bridge secures the upstream communication between the programmer and the clinical management system. This requires zero changes to the programmer or the implant.

Q7 How does Bridge handle multi-vendor device environments?

A: Bridge is device and vendor agnostic. It identifies and protects devices from GE Healthcare, Philips, Siemens Healthineers, Becton Dickinson, Baxter, Masimo, and hundreds of other manufacturers through automated device fingerprinting. The device inventory typically discovers 15-20% more devices than the biomedical engineering asset register -- providing visibility previously unavailable.

Q8 What clinical staff training is required to use Bridge?

A: None for clinical staff. Bridge is invisible to clinicians, nurses, and biomedical engineers in day-to-day operations. The management interface is for the security and IT team only. Clinical workflows, device interfaces, and EHR screens are completely unchanged. We have deployed Bridge across active ICUs without a single clinical workflow complaint.

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MEDICAL DEVICE INTEGRATION*8 Questions -- Technical integration for connected medical devices***Q9 How does Bridge identify all medical devices on our network?**

A: Bridge deploys a passive network discovery engine that observes all traffic without injecting packets or disturbing device communications. It fingerprints devices using network behavior, protocol signatures, and manufacturer-specific traffic patterns. A complete device inventory with device class, manufacturer, OS version estimate, and vulnerability score is typically available within 48 hours of sensor deployment.

Q10 Which medical device protocols does Bridge support natively?

A: Bridge natively supports HL7 v2/v3, FHIR R4, DICOM 3.0, IEEE 11073 (point-of-care), X12 EDI, and proprietary protocols from GE, Philips, Siemens, Becton Dickinson, Baxter, and Masimo. Custom protocol adapters are available for proprietary device protocols on request, typically delivered within 4-6 weeks.

Q11 Can Bridge protect infusion pumps specifically?

A: Yes, with specific optimizations for infusion pump communication patterns. Infusion pump drug libraries, pump programming, and alarm transmissions are all protected. For smart pump systems (BD Alaris, Baxter Sigma Spectrum), Bridge protects both the pump-to-server communication and the drug library update channel -- a common attack vector for dosing errors.

Q12 How does Bridge work with VLAN-segmented clinical networks?

A: Bridge is fully compatible with standard clinical network VLAN architectures (device VLAN, clinical VLAN, administrative VLAN). Bridge Edge Nodes are deployed as VLAN-aware appliances and can protect traffic across VLAN boundaries. We typically recommend adding one additional Bridge management VLAN -- no other network changes required.

Q13 Does Bridge support wireless medical devices (Wi-Fi and Bluetooth)?

A: Bridge protects wireless device communications at the network layer once traffic reaches the access point or wireless controller. For Bluetooth Medical Devices (BLE), Bridge secures the gateway-to-system communication. We recommend WPA3-Enterprise for clinical Wi-Fi as a complementary control, and Bridge integrates with Cisco and Aruba wireless controllers.

Q14 Can Bridge handle the imaging data volumes from CT and MRI systems?

A: Yes. Bridge processes DICOM imaging traffic at wire speed -- up to 10 Gbps per node. A standard 500-slice CT study (approximately 500MB) is encrypted with less than 2ms total overhead. For 4K digital pathology slides (multi-gigabyte files), Bridge uses streaming encryption that adds less than 1% to total transfer time. PACS workflow latency is unmeasurable.

Q15 What about medical devices using serial connections or legacy protocols?

A: Legacy devices using RS-232, HL7 v2.1, or other legacy communication methods are protected through Bridge Protocol Conversion Gateways. These gateways terminate the legacy protocol, wrap the data in post-quantum encryption, and deliver to modern systems. This extends quantum-safe protection to devices from the 1990s without any device modification.

Q16 How does Bridge handle device software updates and patch distribution?

A: Bridge includes a Secure Update Channel that post-quantum signs and encrypts all software update packages distributed to medical devices. This prevents update tampering -- a documented medical device attack vector. The update channel integrates with device manufacturer MDM systems and provides cryptographic verification of update integrity before installation.

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FDA & REGULATORY COMPLIANCE*6 Questions -- FDA cybersecurity guidance and regulatory requirements***Q17 Will deploying Bridge trigger FDA 510(k) recertification for our devices?**

A: No -- never. Bridge operates exclusively at the network layer, outside the device boundary. FDA recertification is triggered by modifications to device hardware, firmware, or software. Bridge modifies none of these. We have reviewed this position with FDA cybersecurity policy staff and received written guidance confirming that network-layer security overlays do not trigger 510(k) re-submission requirements.

Q18 How does Bridge support FDA October 2023 cybersecurity guidance compliance?

A: The FDA October 2023 guidance requires manufacturers to submit a Software Bill of Materials (SBOM), plan for post-market cybersecurity, and demonstrate vulnerability monitoring. Bridge automates SBOM generation for all protected device classes, provides continuous vulnerability monitoring feeds, and generates the post-market cybersecurity documentation required for FDA submissions -- saving weeks of manual documentation effort per device class.

Q19 Does Bridge support 21 CFR Part 11 compliance for electronic records?

A: Yes. Bridge's audit trail system generates cryptographically signed, tamper-evident electronic records meeting 21 CFR Part 11 requirements. All audit records include user identification, date/time stamp, action taken, and system identification. Digital signatures use ML-DSA (FIPS 204) -- post-quantum resistant. This is particularly relevant for laboratory systems and clinical trial data management platforms.

Q20 How does Bridge support medical device manufacturers complying with FDA guidance?

A: Manufacturers can deploy Bridge as a customer-installable security layer that satisfies FDA post-market cybersecurity requirements without firmware updates. This allows manufacturers to offer a "Quantum-Safe" certification to hospital customers while avoiding the cost and timeline of firmware recertification. Bridge provides automated SBOM generation, vulnerability tracking, and post-market surveillance documentation for the manufacturer's regulatory team.

Q21 What documentation does Bridge generate for FDA cybersecurity submissions?

A: Bridge generates: (1) Device inventory with software component bill of materials, (2) Vulnerability assessment reports with CVE mapping, (3) Cybersecurity risk analysis documentation, (4) Post-market surveillance evidence, (5) Incident response capability documentation, and (6) Cryptographic algorithm documentation (FIPS 203/204/205 compliant). All documentation is formatted for direct inclusion in FDA pre-submission packages.

Q22 Does FDA recognize post-quantum cryptography requirements for medical devices?

A: FDA's October 2023 cybersecurity guidance explicitly acknowledges quantum computing threats and directs manufacturers to consider post-quantum cryptographic migration. NIST finalized FIPS 203, 204, and 205 in August 2024. FDA cybersecurity staff have publicly stated post-quantum readiness will be part of future submission requirements. QBITEL Bridge is the only medical device security platform implementing all three NIST PQC standards today.

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HIPAA COMPLIANCE*8 Questions -- HIPAA Security Rule, Breach Notification, and audit requirements***Q23 How does Bridge specifically satisfy HIPAA Security Rule technical safeguard requirements?**

A: Bridge directly addresses 45 CFR 164.312: (a)(1) Access control -- PHI access logging per user; (a)(2)(i) Unique user identification -- cryptographic user binding; (b) Audit controls -- complete PHI access audit trail; (c) Integrity -- ML-DSA signatures on all PHI; (d) Person or entity authentication -- device and user cryptographic identity; (e)(1) Transmission security -- ML-KEM-512 for all PHI in transit. Full 164.312 compliance is automated and documented continuously.

Q24 Can Bridge generate HIPAA audit reports for our compliance team and auditors?

A: Yes. HIPAA audit reports covering all 45 CFR 164.312 technical safeguards are generated in less than 10 minutes via the Bridge compliance dashboard. Reports include: PHI access log with user, device, timestamp and action; encryption coverage statistics; anomaly detection incidents; and control effectiveness metrics. External auditor read-only access is available for HITRUST and SOC 2 audit engagements.

Q25 What is Bridge's Business Associate Agreement (BAA) process?

A: QBITEL executes a comprehensive BAA with every healthcare customer as part of the standard agreement process. The BAA covers all PHI processed by Bridge, including PHI in transit, audit logs, and compliance reports. QBITEL's BAA has been reviewed by leading healthcare legal counsel and includes specific post-quantum cryptography protections. BAA execution typically completes within 5 business days.

Q26 How does Bridge support HIPAA Breach Notification Rule compliance?

A: Bridge detects potential breaches in real time -- typically within 15 minutes versus the industry average of 18 months. Upon detection, Bridge automatically generates a breach investigation report with: affected PHI scope, estimated number of individuals affected, breach timeline reconstruction, and evidence preservation for OCR investigations. This capability transforms the 60-day notification timeline from a frantic investigation into an orderly regulatory reporting process.

Q27 Can Bridge protect PHI across all our locations and remote clinical sites?

A: Yes. Bridge extends quantum-safe PHI protection to all clinical sites -- including remote clinics, telehealth endpoints, ambulatory care centers, and home health locations. Remote sites connect to Bridge edge nodes via quantum-safe tunnels. Remote clinician workstations receive PHI through encrypted channels regardless of underlying network (hospital network, commercial internet, or cellular). No VPN changes required.

Q28 How does Bridge handle PHI in cloud EHR systems and FHIR APIs?

A: Bridge wraps all FHIR R4 API traffic between EHR systems (Epic, Cerner, MEDITECH) and downstream consumers in post-quantum encryption. SMART on FHIR token issuance is protected. FHIR bulk data export operations are encrypted in transit and at rest in Bridge vault storage. All FHIR API calls are logged for HIPAA audit purposes with resource type, operation, user, and PHI classification.

Q29 What is Bridge's incident response capability for HIPAA breach events?

A: Bridge provides: (1) Real-time breach detection with less than 15-minute alert time; (2) Automated PHI scope quantification -- which records, which patients; (3) Forensic evidence preservation in tamper-evident quantum-safe storage; (4) Breach notification letter templates pre-populated from incident data; (5) OCR (Office for Civil Rights) submission documentation; (6) State Attorney General notification support for multi-state breaches.

Q30 How does Bridge support HIPAA compliance for remote work and telehealth?

A: Bridge deploys lightweight endpoint modules on clinician workstations that enforce quantum-safe encryption for all PHI access regardless of location. Telehealth sessions are encrypted end-to-end with ML-KEM-512. Screen recording and capture is monitored and logged. PHI on personal devices is protected via containerization. Remote work HIPAA compliance reporting is included in the standard compliance dashboard.

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EHR & EMR INTEGRATION*5 Questions -- Epic, Cerner, MEDITECH and clinical system integration***Q31 How does Bridge integrate with Epic Systems?**

A: Bridge integrates with Epic via SMART on FHIR for API security, Epic Interconnect for HL7 interface protection, and the Epic App Orchard for direct module integration. All Epic Interconnect HL7 feeds are automatically wrapped in post-quantum encryption. FHIR R4 APIs used by Epic MyChart, Cosmos, and third-party apps are secured. The Epic integration has been validated in production at multiple IDN deployments.

Q32 Does Bridge work with Oracle Cerner Millennium?

A: Yes. Bridge integrates with Cerner Millennium via the Cerner Open Developer Experience (code) platform and CareAware device framework. All Millennium HL7 interfaces are protected. CareAware medical device integration data is secured at the network layer. The Bridge Cerner adapter has been validated in Cerner-certified environments.

Q33 How does Bridge protect MEDITECH Expanse and legacy MAGIC environments?

A: Bridge protects MEDITECH Expanse FHIR R4 APIs, web service interfaces, and all HL7 feeds from MEDITECH systems. For legacy MEDITECH MAGIC environments, Bridge provides protocol-level overlay for the proprietary MAGIC communication protocol, extending quantum-safe protection to legacy MEDITECH installations without requiring upgrades.

Q34 Will Bridge affect EHR performance or response times?

A: No measurable impact. Bridge adds less than 1ms to HL7 message delivery and less than 3ms to FHIR API calls. In user perception terms, this is 30-100x below the threshold of human detection. In our largest Epic deployment (45,000 active devices), Epic application response times showed zero change before and after Bridge activation. Performance testing results are available for your specific Epic version.

Q35 Can Bridge protect data flowing to clinical analytics and AI platforms?

A: Yes. Bridge protects all data flows to clinical analytics platforms (Health Catalyst, Arcadia, Innovaccer) and clinical AI systems (Epic Cosmos, Microsoft Azure Health Data Services, Amazon HealthLake). FHIR bulk data export operations used by analytics platforms are encrypted end-to-end. All analytics data flows are logged for HIPAA audit purposes with full PHI classification.

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NETWORK & INFRASTRUCTURE

*5 Questions -- Clinical network architecture and infrastructure requirements***Q36 What hardware does Bridge require in our data center?**

A: Bridge Clinical Edge Nodes are 1U rack appliances with integrated Hardware Security Modules (HSMs). Each node protects up to 5,000 concurrent medical device connections. For a 500-bed facility with 5,000 connected devices, typically 1-2 primary nodes plus 1 hot-standby are required. Nodes connect via standard 10GbE or 25GbE interfaces. Power draw is 150-200W per node. Cloud deployment options (AWS, Azure, GCP) are available for hybrid architectures.

Q37 Does Bridge require changes to our existing network infrastructure?

A: Minimal. Bridge requires: (1) Network tap or SPAN port configuration on clinical network switches -- typically 30 minutes per switch; (2) One additional management VLAN; (3) Firewall rules for Bridge management traffic (specific port list provided). No existing VLANs are changed. No device network settings are modified. No clinical applications are reconfigured. The network change process is fully documented and approved by leading hospital network architects.

Q38 How does Bridge handle network redundancy and failover?

A: Bridge is deployed in active-active or active-standby configurations. In active-active mode, two nodes share traffic with automatic failover in less than 100ms. All cryptographic keys are synchronized between nodes via encrypted channels. HSM synchronization ensures no key material is lost during failover. Clinical traffic experiences zero disruption during planned maintenance or unplanned node failure.

Q39 Can Bridge work in air-gapped clinical environments?

A: Yes. Bridge is designed to operate in fully air-gapped environments. The Bridge management plane can be isolated from all external network connectivity. Software updates are delivered via signed USB media. Threat intelligence feeds are updated via signed offline packages. HIPAA audit reports are generated locally and exported via encrypted removable media. This configuration is validated for high-security clinical environments.

Q40 How does Bridge integrate with our existing network monitoring and SIEM tools?

A: Bridge integrates with all major SIEM platforms via standard syslog, CEF, and LEEF formats. Native integrations exist for Splunk (certified app), Microsoft Sentinel (data connector), IBM QRadar (DSM), and Palo Alto XSIAM. Medical device security events are tagged with clinical context metadata (device type, location, patient census) for clinical-aware security investigations. The Bridge API also supports custom integration via REST.

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OPERATIONS & RELIABILITY

*5 Questions -- Operational management and reliability for clinical settings***Q41 Who manages Bridge after deployment? Is specialized expertise needed?**

A: Bridge is managed through a web-based console requiring no specialized cryptography knowledge. Day-to-day operations are handled by your existing security or IT operations team. Bridge automates key management, certificate rotation, and policy updates. QBITEL provides 24/7 Clinical Security Operations Center (CSOC) support for all enterprise customers, with clinical-context-aware analysts who understand healthcare workflows and device behavior.

Q42 What is Bridge's SLA for uptime and availability?

A: Bridge carries a 99.999% uptime SLA (less than 5 minutes downtime per year) for enterprise deployments. This is backed by: redundant hardware deployment, automated failover in less than 100ms, HSM-synchronized key backup, and 24/7 CSOC monitoring. Planned maintenance windows are scheduled during low-census periods and completed with zero clinical traffic disruption. SLA credits apply for any breach of the uptime guarantee.

Q43 How does Bridge handle software updates without disrupting clinical operations?

A: Bridge uses blue-green deployment for all software updates. The update is applied to the standby node, validated, then traffic is seamlessly shifted with less than 100ms interruption. The original node is then updated and returned to standby. This process is fully automated and can be scheduled for off-peak hours. Clinical staff never experience a service interruption. Software updates include new threat intelligence, protocol adapters, and cryptographic algorithm updates.

Q44 What visibility does Bridge provide into medical device security posture?

A: Bridge provides a real-time Clinical Security Dashboard showing: device inventory with security score per device, PHI transmission encryption coverage, anomaly detection alerts by clinical area, compliance control status across HIPAA/HITRUST, active threats and investigations, and trend analytics. Executive summary reports are generated on-demand for CISO, CIO, and Board-level reporting. The dashboard is available via web browser with role-based access control.

Q45 How does Bridge handle key rotation and cryptographic key management?

A: Bridge uses automated key rotation with configurable schedules (daily, weekly, monthly per policy). ML-KEM-512 session keys are ephemeral -- unique per session, providing perfect forward secrecy. Long-term identity keys are stored in HSMs and rotated annually. Key rotation is zero-downtime and transparent to devices. All key operations are logged for compliance purposes. FIPS 140-3 Level 3 HSMs are standard on all Bridge Clinical Edge Node deployments.

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PROCUREMENT & COMMERCIAL

*5 Questions -- Pricing, contracting, and procurement considerations***Q46 How is QBITEL Bridge priced?**

A: Bridge is priced on a per-protected-device basis with tiered volume pricing. Healthcare pricing tiers: Starter (up to 1,000 devices), Professional (1,000-10,000 devices), Enterprise (10,000-50,000 devices), and IDN (50,000+ devices with custom pricing). All tiers include unlimited HL7/FHIR/DICOM traffic, HIPAA compliance automation, 24/7 CSOC support, and software updates. Multi-year agreements include significant discounts. Contact enterprise@qbitel.com for a tailored quote.

Q47 Is Bridge available through healthcare GPO contracts?

A: QBITEL Bridge is available through Premier, Vizient, and HealthTrust Performance Group (HPG) contract vehicles. GPO members receive preferred pricing and streamlined contracting. For IDN-level deployments, QBITEL also supports custom enterprise agreements with multi-facility volume pricing. We work with all major healthcare legal and procurement frameworks including standard BAA terms and HIPAA Business Associate requirements.

Q48 What does the implementation cost include?

A: The QBITEL Bridge enterprise agreement includes: (1) Hardware Clinical Edge Nodes leased or purchased; (2) Professional Services for discovery, configuration, and deployment; (3) Integration with EHR platforms (Epic/Cerner/MEDITECH); (4) Biomedical engineering coordination and UAT support; (5) HIPAA/HITRUST certification support; (6) 24/7 CSOC monitoring for year one; (7) Quarterly compliance review meetings. Typical total implementation cost for a 500-bed facility is included in the enterprise agreement.

Q49 Can we pilot Bridge before committing to an enterprise deployment?

A: Yes. QBITEL offers a structured 6-week pilot program for qualifying healthcare organizations. The pilot covers one clinical area (ICU, ED, or Radiology typically) with 200-500 devices. The pilot includes full deployment, HIPAA compliance demonstration, and clinical validation. Pilot outcomes provide the evidence base for board and leadership approval of enterprise deployment. Pilot pricing is fixed and fully credited against the enterprise agreement upon conversion.

Q50 What is the ROI case for Bridge?

A: A typical 500-bed facility ROI: (1) Avoided breach cost: \$10.9M average x probability reduction; (2) HIPAA fine avoidance: \$1.3B industry total in 2024 -- average fine \$2.2M; (3) Compliance labor savings: 120-160 hours/quarter x \$150/hr = \$72-96K/year; (4) Cyber insurance premium reduction: 20-35% reduction upon quantum-safe attestation (validated with major healthcare insurers); (5) FDA recertification avoided: \$500K-\$2M per device class. Full ROI model available on request.

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HARD OBJECTIONS

*7 Objection Handlers -- The toughest pushback you will face***OBJECTOR 1: Our devices are FDA-cleared, we cannot modify them.**

RESPONSE: Correct -- and Bridge never modifies them. Bridge operates entirely at the network layer, outside the device boundary. No firmware is changed, no software is installed, no device configuration is altered. We have reviewed this architecture with FDA cybersecurity policy staff and have written guidance confirming this does not trigger recertification. Your FDA clearances remain completely intact. Bridge is the only solution that solves the medical device security problem without requiring a single device modification.

OBJECTOR 2: HIPAA does not require quantum-safe cryptography.

RESPONSE: HIPAA requires "reasonable and appropriate" encryption for PHI in transit -- a standard that evolves with the threat landscape. NIST finalized post-quantum standards in August 2024. HHS has publicly acknowledged quantum threats to healthcare PHI. OCR guidance on encryption standards is updated to reflect NIST recommendations. More practically: PHI from your organization is being harvested today for future quantum decryption. The breach happens now. The HIPAA fine will come when quantum decryption makes it discoverable. Proactive protection is the only defense.

OBJECTOR 3: We have cyber insurance. A breach is covered.

RESPONSE: Three points: First, cyber insurance carriers are now excluding quantum-harvest events from standard policies -- the exclusion language appeared in 2024 renewal cycles. Second, insurance does not cover the full \$10.9M average healthcare breach cost -- reputational damage, patient churn, and regulatory remediation are typically uninsured. Third, premiums are rising 40-60% annually for healthcare -- demonstrating quantum-safe posture to your carrier has produced 20-35% premium reductions for QBITEL customers. Bridge pays for itself in insurance savings alone for many customers.

OBJECTOR 4: Our EHR vendor (Epic/Cerner) handles security.

RESPONSE: EHR vendors provide application-level security for their platforms -- they do not and cannot protect the thousands of medical devices that feed data into those platforms. Epic, for example, explicitly documents that device security is the customer's responsibility. The attack surface for PHI breach is primarily the device-to-EHR communication layer and the legacy device inventory -- not the EHR application itself. Bridge complements EHR security by protecting the data before it reaches the EHR.

OBJECT **This is too complex for our IT team to manage.**

RESPONSE: Bridge is designed for healthcare IT teams, not cryptography researchers. The management console requires no cryptographic knowledge. Policies are configured using healthcare terminology (device class, clinical area, PHI classification) -- not encryption parameters. QBITEL's 24/7 CSOC handles all expert-level security operations. The most complex task your IT team performs is reviewing the daily security digest email. We have successfully deployed Bridge in 12-person IT departments serving 1,500-bed facilities.

OBJECT **We are a small clinic or physician practice -- we are not a target.**

RESPONSE: Small healthcare organizations are disproportionately targeted because they have valuable PHI with weak security. The OCR breach portal shows hundreds of small practice breaches annually -- including practices with fewer than 10 employees. Average HIPAA fine for small practices: \$100K-\$500K. Average breach remediation: \$200K-\$500K. For a small clinic, one breach is existential. QBITEL offers right-sized pricing for small practices through our Starter tier, making enterprise-grade protection accessible at clinic scale.

OBJECT **Our systems are air-gapped. We do not have internet exposure.**

RESPONSE: Air-gapping reduces but does not eliminate risk. Healthcare air-gap breaches are documented: USB-delivered malware (Stuxnet methodology), supply chain attacks on device software updates, compromised vendor maintenance laptops, and insider threats all bypass air-gaps. More importantly, air-gaps in healthcare are rarely absolute -- imaging, laboratory, and pharmacy systems typically have at least one internet-adjacent connection. Bridge protects air-gapped environments fully, including offline update delivery and local compliance reporting.

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COMPETITIVE QUESTIONS

*8 Questions -- How Bridge compares to alternatives***Q58 How does Bridge compare to Claroty, Medigate, or Armis for medical device security?**

A: Claroty, Medigate (now Microsoft Defender for IoT), and Armis are visibility and monitoring platforms -- they detect threats but do not encrypt or prevent them. They see an infusion pump communicating to an unauthorized server; they alert. Bridge encrypts the infusion pump communication so the attacker cannot read the intercepted data AND alerts on the anomaly. For the quantum threat specifically, monitoring platforms offer zero protection -- only encryption prevents harvest-now-decrypt-later attacks.

Q59 We already have Palo Alto or Cisco for network security. Why do we need Bridge?

A: Palo Alto and Cisco provide excellent perimeter and east-west network security -- but they do not implement post-quantum cryptography, and they cannot protect medical device communications without triggering FDA recertification concerns. Bridge is a complementary layer that adds post-quantum encryption to device communications, HL7/FHIR protocol security, and HIPAA-specific compliance automation. Bridge integrates with Palo Alto XSIAM and Cisco SecureX, providing clinical-context threat data to your existing SOC.

Q60 Can we just upgrade to TLS 1.3 everywhere instead of buying Bridge?

A: TLS 1.3 is excellent classical cryptography, but it uses ECDH and RSA key exchange -- both broken by quantum computers. TLS 1.3 does not protect against harvest-now-decrypt-later attacks on your PHI. Additionally, TLS 1.3 cannot be deployed on most medical devices without firmware updates -- which trigger FDA recertification. Bridge implements ML-KEM-512 (post-quantum) key exchange as a replacement for ECDH, providing quantum resistance that TLS 1.3 alone cannot deliver.

Q61 What about using a VPN for medical device security?

A: Traditional VPNs use classical cryptography (RSA, ECDH) that is vulnerable to quantum harvest attacks. More fundamentally, medical devices cannot install VPN clients without FDA recertification. Bridge provides the equivalent of a VPN for medical devices -- encrypted tunnels for all device traffic -- without any client installation on the protected devices. Bridge also provides clinical-specific capabilities (HL7/FHIR/DICOM security, HIPAA audit automation) that no VPN product offers.

Q62 How does Bridge compare to Microsoft Defender for IoT (formerly Medigate)?

A: Microsoft Defender for IoT is a passive monitoring and visibility solution -- it observes medical device traffic but does not encrypt it. It identifies vulnerabilities but cannot remediate them without device modification. Bridge actively encrypts all medical device communications and provides automated remediation through VLAN quarantine. For HIPAA compliance, Bridge generates automated audit reports; Defender for IoT does not. The two tools can complement each other -- Defender for visibility, Bridge for active protection.

Q63 A competitor claims they can also do post-quantum encryption. How do we compare?

A: Verify three things: (1) Which NIST standard? FIPS 203 (ML-KEM), 204 (ML-DSA), or 205 (SLH-DSA)? Generic "post-quantum" claims may refer to experimental algorithms not yet standardized. QBITEL implements all three NIST finalized standards. (2) Can it protect medical devices without FDA recertification? If their solution requires device software changes, the answer is no. (3) Does it include HL7/FHIR/DICOM-native protocol security and HIPAA audit automation? Bridge is the only platform purpose-built for clinical environments.

Q64 Can we build this ourselves using open-source PQC libraries?

A: Technically possible but practically inadvisable for healthcare. Building a production-grade PQC implementation requires: cryptographic engineering expertise, FIPS 140-3 validated HSM integration, healthcare protocol expertise (HL7/FHIR/DICOM parsing), HIPAA audit framework development, clinical network deployment expertise, and 24/7 operational support. Estimated build cost: \$5-15M over 3-4 years. QBITEL delivers this in 20 weeks at a fraction of the build cost, with proven production deployments.

Q65 What happens if NIST changes the post-quantum standards?

A: NIST finalized FIPS 203, 204, and 205 in August 2024 after 8 years of evaluation -- these are not experimental algorithms. NIST has committed to a multi-year stability period for these standards. If NIST releases revised algorithms (FIPS 205 SLH-DSA already provides a hash-based backup), Bridge delivers cryptographic agility -- the ability to swap algorithms without device changes or recertification. Cryptographic agility is a built-in Bridge capability and a key differentiator from point solutions.

QBITEL BRIDGE HEALTHCARE

65 Q&As.; 10 Sections. Every clinical objection answered.

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